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Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services

Dr. Thomas Keane, M.D., M.B.A.
Assistant Secretary for Technology Policy & National Coordinator
for Health IT
U.S. Department of Health and Human Services

The Honorable Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services

RE: Request for Information: Health Technology Ecosystem (CMS-0042-NC) [RIN 0938-AV68]

Dear Secretary Kennedy, Administrator Oz, and Dr. Keane:

On behalf of AMGA, we appreciate the opportunity to comment on the Request for Information on the Health Technology Ecosystem. AMGA has engaged with CMS and ONC on numerous occasions, particularly around the advancement of value-based care, interoperability, and information blocking. We appreciate the opportunity to continue that dialogue through this comment letter.

Founded in 1950, AMGA is a trade association leading the transformation of healthcare in America. Representing multispecialty medical groups and integrated systems of care, we advocate, educate, innovate, and empower our members to deliver the next level of high-performance health. AMGA is the national voice promoting awareness of our members' recognized excellence in the delivery of coordinated, high-quality, high-value care. Over 177,000 physicians practice in our member organizations, delivering care to more than one in three Americans. Our members are also leaders in value-based care delivery, focusing on improving patient outcomes while driving down overall healthcare costs.

This letter provides AMGA's responses to the Provider and Value-Based Care Organization sections of the Request for Information. Our recommendations reflect the practical experience of organizations operating under both fee-for-service and value-based care models, and address how technology can be harnessed to promote innovation, reduce burden, and improve patient outcomes.

Providers

PR-1: Incentivize Adoption of Digital Tools
Recognizing the growing role of digital tools in promoting health and healthy behaviors, CMS should align payment models to reward the use of products that enable greater care coordination and patient engagement. This would mirror engagement structures seen in Medicare Advantage and commercial insurance, rewarding beneficiaries who take proactive steps to improve their health with direct financial incentives. Additionally, to support providers in the transition to value-based care, existing bonuses for providers participating in Advanced Alternative Payment Models (APMs) should be increased and extended to facilitate the required long-term investment in relevant health technologies. CMS should also make permanent key regulatory telehealth flexibilities, such as the ability to list a practice address for billing, remove frequency limits, and allow for virtual supervision where clinically appropriate.

In addition, as healthcare delivery increasingly incorporates telehealth and mobile technologies, the need for a national physician and provider licensing framework has become more and more pressing. Current state-by-state licensing requirements create significant barriers for patients requiring continuity of care or specialized expertise from their established and trusted care team. A national licensing framework or an expanded interstate compact would streamline this process, enabling physicians and other healthcare providers to practice in multiple states without duplicative licensing processes. A national licensure framework is especially valuable for patients in rural areas or those needing access to specialists—services that are often difficult to obtain in many parts of the country. By enabling care across state lines via telehealth, national licensure would expand access and reduce travel burdens. This reform also reflects the operational reality of many integrated healthcare systems that span multiple states. These systems often concentrate specialists and subspecialists in urban centers or near tertiary hospitals, making them inaccessible via telehealth to patients in neighboring states under current licensing restrictions.

PR-2: Reduce Obstacles to Reporting and Billing with Digital Health Apps
Various Medicare administrative tasks, including electronic clinical quality measure (eCQM) submission, Annual Wellness Visit (AWV) documentation, and prior authorization appeals, could be simplified through automation and standardization. CMS should adopt EHR-integrated templates for AWV compliance and expand capabilities for automated population of eCQMs.

CMS should also promote consolidated submission portals and reduce administrative redundancy by aligning submission requirements across quality programs. A centralized, single interface reporting system with parallel requirements for commercial payers would allow quality data submission across all programs, promoting interoperability and reducing system-wide burden.

AMGA applauds CMS's recent efforts to streamline existing quality measure sets through the Universal Foundation, which prioritizes patient-centered, outcome-based measures with simplified and standardized reporting requirements. Notably, the AMGA Value Measure Set served as a precursor to this initiative. It reflects core measures that matter to patients and providers, emphasizes health outcomes over process, and reduces administrative complexity by limiting the number of required measures while maximizing their impact. These enterprise-level measures evaluate performance across an entire health system or medical group, rather than at the individual clinician level, allowing for a more accurate and meaningful assessment of quality in team-based and coordinated care environments. By focusing on system-wide outcomes, enterprise measures incentivize collaboration, reduce administrative burden, and better align with value-based care goals. AMGA believes that widespread implementation of these measures is essential to drive continuous improvement, ensure

accountability, and ultimately enhance patient outcomes across diverse populations. CMS should continue building on this foundation by adopting harmonized, meaningful metrics that improve care delivery without overwhelming frontline clinicians.

To this end, we further recommend that CMS use existing authority under the IMPACT Act to establish a unified post-acute care (PAC) payment framework that consolidates documentation and billing requirements across inpatient rehabilitation facilities (IRFs), skilled nursing facilities (SNFs), long-term care hospitals (LTCHs), and home health agencies (HHAs). Currently, these settings operate under disparate statutory and regulatory rules with separate assessment tools (e.g., IRF-PAI, MDS, OASIS), inconsistent quality reporting systems, and payment methodologies that create complexity for multi-setting providers and care transitions.

PR-3: Expand Interoperability

CMS should require that EHR vendors meet a minimum standard for Fast Healthcare Interoperability Resources (FHIR) Application Programming Interface (API) functionality and compatibility. Ensuring that claims and clinical data are structured and exchangeable will streamline care delivery and reporting while reducing duplicative data entry.

PR-4: Establish Minimum Data Quality Standards

AMGA recommends establishing minimum quality standards for data exchange, which ONC's existing TEFCA framework does not explicitly mandate. Rather, under the Trusted Exchange "Standardization" principle, ONC uses broad language to describe the expectation that Health Information Networks (HINs) meet quality standards:

> *"HINs should, <u>to the extent possible</u>, ensure that the data exchanged within their own network and with other HINs meets minimum quality standards by using testing and onboarding programs to verify minimum quality levels."*[1]

CMS and ONC should establish baseline security, privacy, and functionality requirements for third-party digital tools that access EHR data, to ensure safe use while preserving flexibility. Certification pathways should be streamlined to avoid discouraging innovation.

PR–5 to PR–8: Promote FHIR Tools and Reduce Data Burden

AMGA members report increasing use of FHIR APIs for bulk data export, care coordination, and quality reporting. These tools have enabled providers to automate and streamline many workflows that were previously manual and labor-intensive. As AMGA members shift toward value-based care, access to standardized, real-time data is essential to proactively manage risk, engage patients, and demonstrate value.

Accordingly, we make the following recommendations to promote FHIR tools:

- CMS should expand tools such as Blue Button and prior authorization APIs and encourage commercial payers to offer comparable capabilities. This would enhance care coordination while improving transparency and accountability in payer-provider relationships.
- To promote TEFCA implementation and ensure consistent nationwide data exchange, ONC and CMS should include more explicit standards that ensure provider access to commercial claims data. These

[1] Trusted Exchange Framework and Common Agreement (TEFCA) https://www.healthit.gov/sites/default/files/page/2022-01/Trusted_Exchange_Framework_0122.pdf

standards should also include enforceable requirements for data completeness, timeliness, and usability, helping providers avoid gaps in patient records that compromise care management and quality reporting.

To reduce administrative burden, we emphasize again that CMS should consolidate overlapping or duplicative reporting requirements across its quality and utilization programs. A single reporting interface with aligned timelines, data specifications, and validation rules, would ease compliance and reduce costs for providers operating across multiple federal programs. When paired with interoperable FHIR-based reporting infrastructure, providers can report on a streamlined set of quality measures using automated performance tracking that is seamlessly integrated into clinical workflows.

PR-12: Revise Information Blocking Exceptions

AMGA strongly supports ASTP/ONC's creation of the Protecting Care Access Exception and urges the establishment of an additional Requestor Preferences Exception. While the current information blocking regulations aim to increase transparency, in practice they have created confusion and unintended consequences within the doctor-patient relationship by requiring immediate access to electronic health information, regardless of patient preference or clinical context.

AMGA members have reported troubling cases where patients were notified of distressing test results before any communication with a provider could occur. These incidents have led to emotional harm and patient dissatisfaction, particularly when they occur after hours or over weekends with no provider available to explain the results.

A Requestor Preferences Exception would allow patients to express how and when they wish to receive certain information, such as delaying lab results until they can review them with their physician. This would promote more personalized, compassionate care without placing providers at risk of being labeled information blockers.

To support this change, AMGA also recommends expanding the definition of "harm" within the information blocking framework to include emotional and psychological harm. This broader definition would give clinicians appropriate discretion to delay the release of sensitive results when there is a legitimate risk of distress, thus better aligning policy with real-world care delivery.

PR-13-14: Fair Enforcement of Information Blocking Rules

Regarding enforcement of information blocking rules, AMGA recommends that CMS develop a unified oversight framework across CMS, ONC, and OCR to avoid duplicative audits and conflicting standards. Today, each agency imposes overlapping or unclear requirements on providers, which leads to confusion, inefficiencies, and increased administrative burden. A streamlined model that integrates responsibilities and creates a single pathway for compliance inquiries and resolution would greatly improve provider understanding and adherence.

Regarding enforcement of information blocking rules, we advocate that the 2024 Final Rule entitled, "21st Century Cures Act: Establishment of Disincentives for Health Care Providers That Have Committed Information Blocking" (45 CFR § 171; RIN 0955–AA05; 21st Century Cures Act), be repealed.

Finalized under the previous administration, the rule titled *"21st Century Cures Act: Establishment of Disincentives for Health Care Providers That Have Committed Information Blocking"* imposes substantial penalties on Medicare-enrolled providers accused of information blocking. While promoting data sharing is a critical goal, the regulation is both overly punitive and poorly structured. The penalties are excessive, and the final rule disregarded industry feedback requesting a more balanced approach that includes provider education,

technical assistance, and the opportunity to implement corrective action plans prior to enforcement.

Providers acting in good faith to comply with information blocking rules should be protected. CMS should issue guidance to clarify that providers who make reasonable efforts to interpret and apply the regulations, especially in cases involving complex or conflicting privacy and safety considerations, will not face immediate punitive measures.

Additionally, CMS should reinforce protections for whistleblowers and internal compliance staff who identify or report potential violations, ensuring they are shielded from retaliation and included as part of a constructive compliance improvement process.

VALUE-BASED CARE ORGANIZATIONS

VB–1: Incentives for ACO Digital Tool Adoption

To increase adoption of digital tools by ACOs, CMS should strengthen pathways for allowable expenditures on digital infrastructure (including care management platforms, remote monitoring systems, and analytics tools) to count toward shared savings and loss calculations. Many ACOs must forgo investments in these technologies due to uncertainty about return on investments and limitations on how such costs are treated in financial reconciliation. Revising financial benchmarking methodologies to recognize the long-term value of these digital investments would improve management of population health, therefore reducing costly downstream utilization.

Additionally, CMS should improve the payment structure for remote patient monitoring (RPM). Under current guidelines for CPT® 99454, if a patient transmits 15 values in a 30-day period, the provider receives no reimbursement—however, if the patient transmits 16 or more, the provider may bill.[2] This rigid threshold can disincentivize the use of RPM and fails to recognize the clinical value of more limited patient engagement.

Access to complete and timely commercial claims data is another critical enabler of successful ACO performance. While CMS has made progress on Medicare data availability, commercial payer data—especially for out-of-network services—remains difficult to access. CMS should require commercial payers participating in federal programs to make standardized claims feeds available via APIs to contracting ACOs and providers.

Finally, the 5% APM bonus remains a key incentive for participation in advanced payment models. CMS should work with Congress to extend and stabilize this bonus to encourage long-term transformation. Technical assistance, learning collaboratives, and implementation grants should also be made available to small and rural ACOs that lack the internal infrastructure to adopt new tools without external support.

VB–2 to VB–4: AI and Analytics Integration

Artificial intelligence (AI) and advanced analytics must be thoughtfully embedded into clinical workflows to be effective. CMS should support the integration of AI-driven tools that assist with patient risk stratification, medication adherence monitoring, and proactive outreach for gaps in care. Importantly, these tools should be accessible through EHR-integrated applications built using standardized FHIR-based APIs to ensure usability across vendor platforms.

[2] CPT® is a registered trademark of the American Medical Association.

Essential technologies for value-based care include real-time risk stratification engines that can identify rising-risk patients before hospitalization, allowing for timely intervention and care planning. Equally important are tools that integrate claims and clinical data to consolidate fragmented information into a single, actionable source for providers. In addition, patient-reported outcomes platforms support longitudinal tracking of functional status and well-being, offering critical insights into treatment effectiveness and quality of life over time.

CMS should consider funding pilots or implementation guides that support the safe and responsible deployment of such tools in diverse care settings.

As artificial intelligence becomes more deeply integrated into healthcare delivery, the need for robust cybersecurity infrastructure becomes increasingly urgent. Health systems and providers face rising threats from sophisticated cyberattacks that pose serious risks to both patient privacy and jeopardize critical clinical operations. It is essential that Medicare and other public payers evolve to support providers' cybersecurity readiness. This includes aligning payment models and regulatory incentives to help offset the high costs of securing AI-enabled technologies and protecting sensitive data. Without such support, the promise of AI to improve care quality and reduce costs could be undermined by growing digital vulnerabilities and operational risks.

VB–5 to VB–7: Certification Reforms

The current ONC certification framework remains largely designed for fee-for-service billing and documentation. For providers in value-based care arrangements, this framework does not sufficiently support care coordination, team-based care, or longitudinal population management. AMGA recommends that ONC develop a parallel certification track focused on value-based capabilities.

Ideally, a new certification pathway should emphasize care management, team communication, and integration of claims and SDOH data. Additionally, it should include modular certification to reduce the burden on providers who already have some tools in place. Finally, the pathway should allow for alternative demonstration of capability through attestation or third-party validation for smaller practices.

We emphasize that certification should not force ACOs into costly EHR upgrades that are unrelated to clinical function or patient benefit. Instead, CMS and ONC should ensure certification requirements are aligned with the operational realities of population health management.

VB–8 to VB–10: Flexible Standards for Smaller Providers

Small practices are a vital part of the health system, but they often lack the resources and infrastructure to comply with one-size-fits-all digital health mandates. AMGA recommends that CMS adopt tiered requirements to reflect differences in provider size, location, and capabilities. For example, flexibility around the timing and scope of FHIR integration and EHR adoption should be built into any future rulemaking.

ACOs and independent physician groups should not be forced to adopt enterprise-wide EHR systems solely to meet certification or reporting requirements. CMS should promote function-specific, modular certification pathways that allow providers to adopt targeted digital capabilities without incurring the costs of comprehensive system overhauls. This approach would expand participation in value-based models and reduce disparities in digital infrastructure between rural and urban providers.

VB–11 to VB–15: Interoperability and Directory Improvements

Timely, standardized access to claims and clinical data remains foundational to success in APMs. CMS should

require all participating payers—including Medicare Advantage and commercial carriers—to provide structured claims data via APIs, with defined update intervals. This will allow providers to perform timely care management, trend analysis, and attribution validation.

At the same time, innovation must not come at the expense of baseline compatibility. CMS and ONC should require all certified EHR vendors to support a core set of FHIR APIs aligned with USCDI standards. This will allow developers to build plug-and-play applications that function consistently across systems.

Finally, a national FHIR-based provider directory would be transformative for the healthcare ecosystem. It would enable cleaner attribution for both quality measurement and financial performance, ensuring that patients are accurately aligned with their providers. Additionally, it would simplify credentialing and contracting processes for value-based networks, streamlining operations and improving efficiency. By centralizing and standardizing provider information, this directory would also reduce administrative burden for payers, providers, and patients.

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We thank you for your consideration of our comments. Should you have questions, please do not hesitate to contact AMGA's Darryl M. Drevna, senior director of regulatory affairs, at 703.838.0033 ext. 339 or at ddrevna@amga.org.

Sincerely,

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